Proposed IEHP Case Strategy

Important: We should not lock the final thesis before analyzing the data

We should begin with a provisional thesis, use the data to test it, and then finalize the thesis based on what the evidence actually shows.

Our starting hypothesis is:

> The FFS transition creates two connected risks for IEHP: vulnerable members may lose practical access to care, while IEHP may be left with stranded fixed costs and uncertain financial exposure.

We should test this by analyzing:

- Which ZIP codes or cities have the weakest FFS provider access
- Whether the largest gap is primary care, specialty care, mental health, or dental care
- Which patient groups are most vulnerable to losing continuity
- Whether the financial risk is driven mainly by lost revenue, fixed costs, or patient mix
- Whether healthier or sicker members create the greater financial impact

The final thesis should reflect the data. For example, if the data shows that specialty access is the biggest issue, our recommendation should focus more heavily on specialty continuity. If the worst gaps are concentrated in a few ZIP codes, our solution should target those areas instead of treating the entire Inland Empire equally.

Our provisional central idea

> IEHP should build an “Access Bridge” that protects patient continuity, identifies where access is most likely to fail, and helps IEHP financially adapt to the loss of members.

The Access Bridge would contain three connected recommendations:

1. A portable Care Passport for high-risk members
2. An Access Gap Engine that identifies where IEHP should intervene
3. A Financial Risk Simulator that connects transition scenarios to specific IEHP actions

These recommendations are our starting framework. The data should determine how we rank them and which patient groups, services, and locations we emphasize.

Recommendation 1: Medi-Cal Care Passport and Warm Handoff Network

Problem

When members move from IEHP managed care to FFS, they may lose:

- Their assigned PCP
- Referrals
- Care-management relationships
- Medication continuity
- Knowledge of their medical history
- Access to ECM and Community Supports

Solution

IEHP would provide high-risk members with a portable Care Passport containing:

- Current doctors
- Active referrals
- Medications
- Upcoming appointments
- Current treatment needs
- Preferred language
- Transportation or communication preferences
- Instructions for finding an FFS provider

IEHP would also conduct a warm handoff by helping connect the member to an FFS provider, clinic, or community partner.

Why it is distinctive

This is more than “improve care coordination.” It creates an actual tool that carries the member’s care information across the payment-system transition.

Prototype

Create a fictional Care Passport mockup showing:

1. The member's view
2. The provider's view
3. The warm-handoff process

Metrics

- Percentage of high-risk members receiving a Care Passport
- Percentage with a verified provider or appointment within 30 days
- Medication continuity rate
- Referral completion rate
- Emergency-department visits after transition

Privacy safeguards

- Do not display UIS or immigration status.
- Use only the medical information necessary for continuity.
- Use secure, role-based access.
- Allow members to choose safe contact methods.
- Require privacy and legal review before implementation.

Recommendation 2: Access Gap Engine and Provider Capacity Exchange

Problem

The FFS provider file shows who is enrolled, but enrollment does not necessarily mean that the provider:

- Accepts new patients
- Has available appointments
- Is geographically convenient
- Provides the needed specialty

Solution

Build an explainable Access Gap Engine that ranks ZIP codes or cities by access risk.

It could analyze:

- Medi-Cal members in the area
- FFS primary-care providers

- FFS specialists
- Mental-health providers
- Dental providers
- Providers per 1,000 members
- Appointment availability
- Travel distance
- Shortage-area indicators
- Concentration of high-risk members

The system should produce an action recommendation, not just a map.

For example:

> This ZIP code has high Medi-Cal demand, low FFS primary-care supply, and long appointment wait times. IEHP should send provider-enrollment support and reserve continuity appointments in this area.

Why it is distinctive

Other groups may recommend a dashboard. Our dashboard would go further by connecting each high-risk location to a recommended IEHP action.

Prototype

Create three dashboard views:

1. Regional heat map
2. Provider capacity table
3. Action queue showing the owner, action, and deadline

Metrics

- Reduction in high-risk ZIP codes
- FFS providers successfully enrolled
- Average appointment wait time
- Percentage of providers accepting new patients
- Providers per 1,000 Medi-Cal members
- Member access complaints

Privacy safeguards

- Use ZIP-level or city-level aggregate data.
- Do not show member names, addresses, or individual medical records.
- Suppress small populations that could be re-identified.

- Do not use race, ethnicity, foreign-born status, or other demographic variables to infer UIS status.
- Use the model to direct resources, never to deny or reduce care.
- Include human review before taking action.

Recommendation 3: Financial Risk Simulator and Trigger-Based Cost Plan

Problem

IEHP may lose a large amount of revenue, but lost revenue does not equal lost profit. IEHP will also stop paying many medical costs for members who leave.

The larger financial concern may be stranded fixed costs, including:

- Staff
- Technology
- Finance
- Facilities
- Administration
- Provider-support infrastructure

The type of members leaving also matters. Healthier members leaving could create a much larger financial loss than sicker members leaving.

Solution

Build a Financial Risk Simulator using the provided workbook.

It should test:

- Low, middle, and high affected-member scenarios
- Healthier, base, and sicker patient mixes
- Revenue lost
- Medical costs avoided
- Variable operating costs avoided
- Fixed costs remaining
- Net financial effect
- Potential increases in emergency or hospital use

Then connect each scenario to a management action.

Scenario	IEHP response
Healthier members leave faster than expected	Protect retention and quality programs
Fixed costs become too high per remaining member	Redesign staffing and administrative capacity
High-risk members remain in IEHP	Redeploy care-management resources
Emergency visits increase after transition	Escalate evidence to DHCS and invest in access support

Why it is distinctive

Instead of only presenting financial estimates, we would create a decision system that tells IEHP what to do when conditions change.

Prototype

Create a scenario dashboard showing:

- Low, middle, and high cases
- Healthier, base, and sicker cases
- Net IEHP impact
- Fixed costs remaining
- Recommended action

Metrics

- Administrative cost per remaining member
- Fixed cost per remaining member
- Net operating margin
- Care-management capacity
- Access and quality measures
- Emergency utilization after the transition

Privacy safeguards

- Use aggregate financial and utilization scenarios.
- Do not identify individual “profitable” or “unprofitable” members.
- Do not reduce care for expensive or high-risk members.
- Clearly label assumptions and ranges.
- Use the model for planning, not treatment or eligibility decisions.

Data collection plan

We should not collect data just to prove these recommendations. We should collect data to determine whether these recommendations are actually the right response.

Access data

Use the sources listed in the Access to Care guide:

1. Medi-Cal members by ZIP code
2. Medi-Cal FFS providers
3. ZIP-to-county crosswalk
4. Managed-care enrollment by county and plan
5. Optional HRSA shortage-area data

Process

Download data
↓
Filter to Riverside and San Bernardino
↓
Clean ZIP codes
↓
Remove duplicate providers
↓
Classify provider types
↓
Count members and providers by ZIP
↓
Calculate providers per 1,000 members
↓
Rank the highest-risk areas
↓
Revise the thesis based on findings

Formula:

FFS providers per 1,000 members
=
FFS providers ÷ Medi-Cal members × 1,000

Do this separately for:

- Primary care
- Specialists
- Mental health
- Dental

Financial data

Use the provided Financial Impacts Modeling workbook to analyze:

- Low, middle, and high affected-member scenarios
- Healthier, base, and sicker patient mixes
- Revenue lost
- Medical costs avoided
- Fixed costs remaining
- Net IEHP impact

We should also explain the difference between the fact-sheet enrollment figure and the September 2026 county enrollment figure rather than pretending they are identical.

What we should build for the final presentation

The official deliverable is the five-minute video. Our supporting prototypes should appear inside the video.

We should create:

1. A Care Passport mockup
2. An Access Gap Engine dashboard or map
3. A financial scenario dashboard
4. A provider action queue
5. A privacy and equity guardrails slide

We do not need to build a production healthcare application. A polished prototype using fictional or aggregate data is enough.

Possible presentation structure

Slide 1: The answer

> IEHP needs an Access Bridge, not just a transition plan.

Slide 2: The problem

Explain the difference between coverage, provider enrollment, and actual appointment access.

Slide 3: Data findings

Show the ZIP codes, specialties, or patient groups that the data identifies as most vulnerable.

Slide 4: Financial impact

Show revenue lost, medical costs avoided, fixed costs remaining, and net impact.

Slide 5: Care Passport

Show how patient continuity would be preserved.

Slide 6: Access Gap Engine and Financial Simulator

Show the dashboard, action queue, and scenario triggers.

Slide 7: Implementation and safeguards

Show owners, timeline, metrics, privacy protections, sources, and AI use.

Short version of the pitch

> Our team should recommend that IEHP create an Access Bridge for the 2027 FFS transition. The Access Bridge would include a Care Passport that preserves continuity for high-risk members, an Access Gap Engine that identifies the ZIP codes where FFS access is most likely to fail, and a Financial Risk Simulator that connects different member and patient-mix scenarios to specific operating actions. We would begin with these as hypotheses, analyze the member, provider, and financial data, and then finalize the thesis based on what the evidence shows. This makes our proposal data-driven instead of forcing the data to support a recommendation we chose in advance.

IEHP Consulting Project — Simplified Action Plan

1. What is the main problem?

IEHP is losing members to Fee-for-Service (FFS), but those members may lose access to their existing healthcare providers. At the same time, IEHP will lose revenue and must adjust its operations.

Your job as a consultant is to answer three questions:

1. Where is the problem worst? Identify communities with many affected members but limited FFS providers.
2. How much will it impact IEHP? Estimate financial losses and operational risks.
3. What should IEHP do? Recommend measurable actions that IEHP can realistically implement.

The deliverable should not just be an explanation. It should demonstrate how IEHP can use data to make better decisions.

2. Proposed solution: IEHP Transition Risk Dashboard

IEHP Transition Shield

An interactive dashboard that identifies communities at risk of losing healthcare access and shows how IEHP could reduce that risk.



What makes this presentation-worthy?

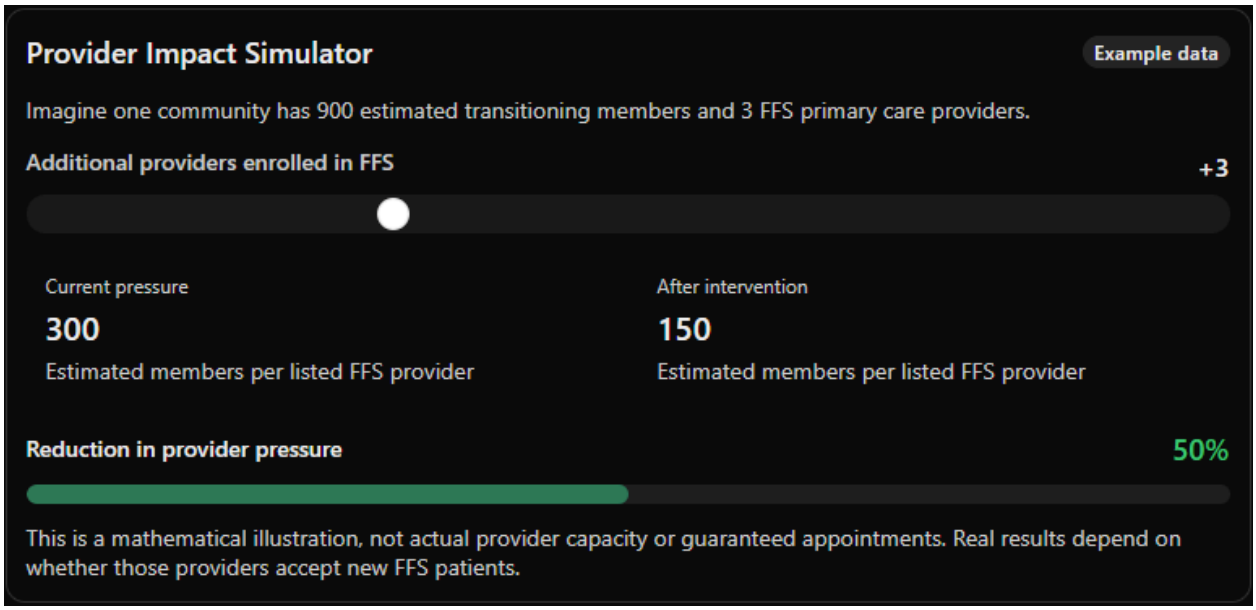
You demonstrate a simple decision-making process:

1. Identify: Show the ZIP codes with the greatest estimated shortage of FFS providers.
2. Act: Select a high-risk area and simulate helping additional doctors enroll in FFS.
3. Measure: Show how the estimated provider shortage changes and how many additional patients those doctors might serve.

The creative element is a "What-If Provider Simulator."

For example:

Provider Impact Simulator



This can be replicated in Power BI using a What-If parameter, in Tableau using a parameter, or in Python using Streamlit.

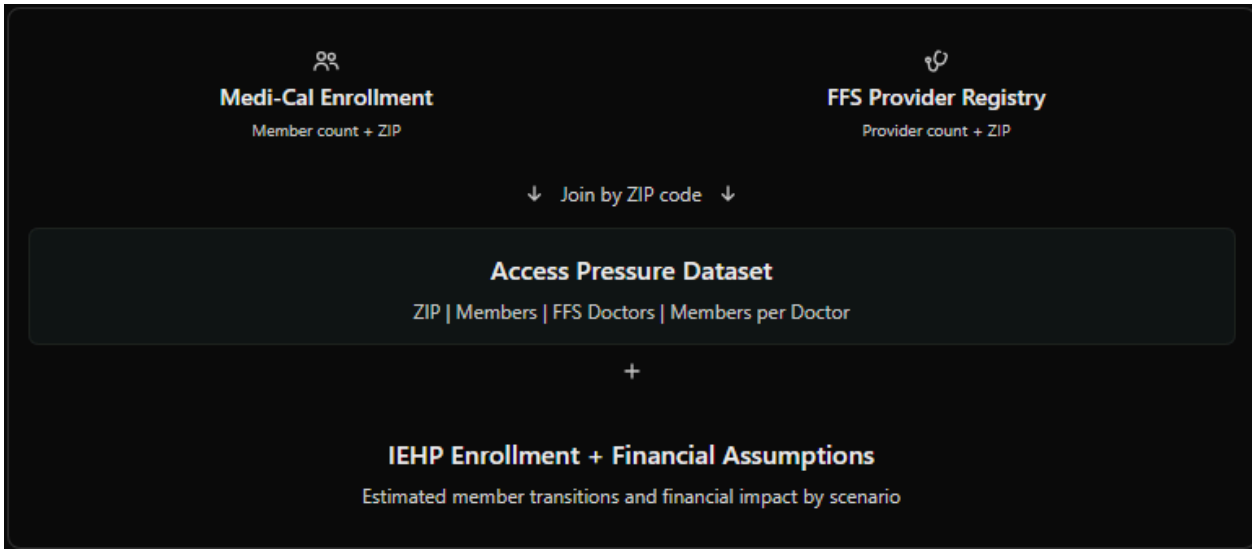
3. What data do we need?

Start with only four datasets.

Dataset	Information needed	Purpose
A. Medi-Cal members by ZIP	ZIP code, number of members	Measure geographic demand
B. FFS providers	ZIP code, provider ID, specialty	Measure available provider supply
C. IEHP enrollment	Members by county and plan	Estimate number transitioning
D. Financial model	Revenue, medical expenses, fixed costs	Estimate financial impact

The first three are available through the linked public sources in your Access to Care guide. The fourth is already in the supplied Excel workbook. “CEO Business Challenge Student Guide Access to Care Analysis.pdf”

How to connect the datasets:



Important: Total Medi-Cal enrollment by ZIP is not the same as the number of UIS members transitioning. Use it as a measure of general healthcare demand. For transition-specific estimates, use the case guide's 10%, 14%, and 18% scenarios and clearly label any geographic allocation assumptions. “CEO Business Challenge Student Guide Access to Care Analysis.pdf”

For a more accurate version later, add IEHP's internal provider network, actual affected-member counts, and information about which providers are accepting new patients.

4. What should we measure? One main metric

Transition Provider Pressure (TPP)

This is the metric I would build the project around.

$$\text{TPP} = \frac{\text{Estimated transitioning members}}{\text{FFS primary care providers}}$$

Interpretation: How many potentially transitioning members correspond to each listed FFS primary care provider in an area?

- Higher TPP = potentially greater access pressure.

- Lower TPP = potentially more provider availability.
- Zero FFS providers = flag as no locally listed provider, rather than dividing by zero.

This metric connects member demand and provider supply into one easy-to-understand number.

It does not prove how many patients each doctor can actually accept. Use it to identify locations for further investigation.

Additional useful metric: Provider Conversion Opportunity.

Estimate how much TPP would decrease if IEHP helped one or more existing network doctors enroll in FFS. This turns your dashboard from a description of the problem into a decision tool.

5. The exact dashboard graphs I would create

Build these five visuals, in this order.

Graph	X / Y or fields	Story it tells
1. ZIP Code Risk Map	ZIP boundaries colored by TPP or general provider availability	Where are the highest-risk communities?
2. Top 10 At-Risk ZIPs	X: TPP, Y: ZIP code; separate labels for ZIPs with zero providers	Which locations deserve attention first?
3. Demand vs. Supply Scatter Plot	X: estimated transitioning members, Y: number of FFS PCPs; each dot is a ZIP	Which areas have high demand and few providers?
4. Financial Waterfall	Lost revenue → avoided medical costs → avoided operating costs → net impact	Why does losing revenue not equal losing that much profit?

5. Before-and-After Intervention	X: chosen ZIP or scenario, Y: TPP before and after provider enrollment	What improvement could IEHP achieve?

The key storytelling visual is Graph 3. Communities with high projected demand and few providers should appear in the lower-right part of the scatter plot. These become potential intervention targets.

The key demonstration is Graph 5, where you change the number of additional FFS providers and show the updated result.

6. KPIs to put at the top of the dashboard

I would show only four main KPIs.

KPI	Why it matters
Projected Members Transitioning	Shows the size of the challenge; label scenario assumptions
High-Risk ZIP Codes	Shows how many areas exceed a chosen access-pressure threshold
Provider Readiness Rate	% of IEHP primary care providers confirmed FFS-enrolled; requires IEHP network data
Estimated Net Financial Impact	Shows potential annual change to IEHP's operating result

For ongoing monitoring after January 1, add two outcome measures if IEHP can obtain the necessary data: successful care handoff rate and time to obtain a primary care appointment.

These are important because a lower TPP does not necessarily mean patients are successfully receiving healthcare.

7. Break the entire project into five achievable tasks

Task 1 — Build a clean dataset

Download ZIP-level Medi-Cal enrollment and FFS provider records. Filter to Riverside and San Bernardino counties and count primary care providers by ZIP.

Deliverable: One Excel or CSV table containing ZIP, members, and FFS doctors.

Task 2 — Calculate access pressure

Calculate members per FFS provider for every ZIP, rank areas, and identify ZIPs with no listed providers. Add modeled transition demand as a separate scenario field.

Deliverable: Ranked ZIP-level risk table.

Task 3 — Create the core visualizations

Build a ZIP map, Top 10 risk chart, and demand-versus-supply scatter plot.

Deliverable: An interactive access-risk dashboard.

Task 4 — Add the provider simulator

Allow users to choose a community and simulate the impact of enrolling 1–10 additional providers.

Deliverable: Live before-and-after demonstration using clearly stated assumptions.

Task 5 — Add financial impact and recommendations

Use the existing Excel workbook to show low, middle, and high financial scenarios. Connect your findings to three proposed actions.

Deliverable: Financial waterfall, executive KPIs, and final recommendations.

Keep the first version limited to primary care, the two IEHP counties, and one recent month of data. Expanding to specialty care, dental, and mental health can be a future improvement.

8. What do you recommend to IEHP?

Your final recommendations can remain very simple:

Priority	Recommendation	What IEHP does
1	Target high-risk communities	Use the dashboard to identify locations needing the most provider and member support
2	Help providers transition to FFS	Contact existing network doctors who are not FFS-enrolled and assist with the transition process
3	Protect financial stability	Monitor membership losses, medical expenses avoided, and remaining fixed costs

For the five-minute presentation, demonstrate Priority 1 and Priority 2 together using the dashboard and simulator. Explain Priority 3 with the financial waterfall. This aligns with the rubric's emphasis on evidence-backed, actionable recommendations.
“CEO_Challenge__Rubric.docx”

The message you want IEHP's CEO to remember:

> We developed a data-driven tool that identifies where patients are most likely to face healthcare access problems and helps IEHP determine where provider outreach could have the greatest impact.

That is a focused consulting project with a measurable problem, a working demonstration, and a clear business recommendation.

Objective: advise IEHP on how to respond to the January 1, 2027 movement of UIS Medi-Cal members from Managed Care to FFS by estimating the effects on IEHP, patients, and providers, then recommending priorities and monitoring metrics in a 5 min presentation

Slides:

Opening problem statement
What is changing, and why should IEHP care?
Background and timeline
Explain Managed Care vs. FFS, what changes on January 1, 2027, and who is affected.
What is at stake for IEHP
Brief SWOT, but make every strength, weakness, opportunity, and threat specific to this transition.
Your analysis and insights
Show the effects on:

- Patients
- Providers
- IEHP finances and operations
- Vulnerable geographic or population groups

Data-supported evidence
Present your strongest numbers, charts, maps, scenarios, and assumptions. This should support your insights rather than feel like a separate data dump.
Recommendations to IEHP
What should IEHP do immediately, longer term, and possibly advocate for with the state?
Progress tracking and timeline
Show what IEHP should monitor before, during, and after the transition.

Sources and AI disclosure

Put source labels near the relevant numbers. Include a reference slide or reference sheet, then end with the required AI-use explanation slide.

Data questions

Question	Data to examine	Important cleaning decisions	Source
How many members may be affected?	IEHP Medi-Cal enrollment by county and scenario percentages	Separate actual enrollment from assumed UIS percentages	D / FI-1
Where might access gaps occur?	Medi-Cal members by ZIP code and FFS providers by ZIP code	Store ZIP codes as text, filter to the two counties, remove duplicate provider locations	A E / FI-10 H
Who may be most vulnerable?	County-level age, language, and plan data	Use this to understand vulnerability, not to estimate UIS status	B
What is the financial impact?	Financial Impact Estimate and Segment & Cost Scenarios workbook tabs	Separate sourced numbers from assumptions; distinguish lost revenue from net financial impact	D / FI-1 FI-7
How should IEHP track progress?	Enrollment, provider, access, and financial data	Establish a baseline before January 1, 2027, then compare afterward	A B D / FI-1 E / FI-10 FI-7

IEHP — Data Analysis Objectives

Goal: Identify the biggest healthcare access and financial risks from the 2027 FFS transition.

1. Provider Comparison

- Count IEHP-only, FFS-only, and overlapping providers.
- Identify specialties and ZIP codes with the lowest overlap.

2. Healthcare Access Gaps

- Find ZIP codes with high Medi-Cal demand but few FFS providers.
- Rank the top 5–10 highest-risk areas.

3. Healthcare Services

- Determine whether primary care, specialty care, mental health, or dental faces the biggest shortages.
- Check whether FFS providers accept new patients, if data is available.

4. Vulnerable Patients

- Identify age groups, language needs, and patients requiring continuous treatment.

5. Financial Impact

- Estimate revenue lost, costs avoided, and fixed costs remaining.
- Compare low, medium, and high transition scenarios, including different patient health profiles.

6. Final Recommendations

Use the findings to determine:

- **Where** IEHP should intervene.
- **Who** needs the most assistance.
- **What** healthcare services need priority.
- **How** IEHP should respond financially and operationally.

Final outcome: A provider comparison, access-risk map, financial analysis, and targeted action plan.

Note: Provider enrollment does not guarantee appointment availability, and all estimated impacts must be labeled as assumptions.

IEHP Dashboard — Final Data We Need

I'll be pulling these numbers from the datasets for Riverside and San Bernardino counties:

1. Provider Comparison

- Total IEHP providers and FFS providers
- Number of IEHP-only, FFS-only, and overlapping providers
- Percentage of IEHP providers also enrolled in FFS
- Breakdown by ZIP code and specialty

2. Healthcare Access

- Total Medi-Cal members per ZIP code
- FFS providers per 1,000 Medi-Cal members
- Top 5–10 ZIP codes with the lowest provider availability
- Specialties with the greatest potential shortages

3. Financial Impact

- Low, middle, and high affected-member scenarios
- Revenue lost, medical costs avoided, and fixed costs remaining
- Net financial impact under different patient health scenarios

How I'll Process the Data

1. Filter everything to Riverside and San Bernardino.
2. Match IEHP and FFS providers using NPI or verified identifying information.
3. Remove duplicates and calculate provider overlap.
4. Combine provider counts with ZIP-level Medi-Cal populations.
5. Calculate provider ratios and rank high-risk areas.
6. Extract financial scenario results from the provided workbook.

Final Dashboard Outputs:

- Provider overlap comparison
- Healthcare access heat map
- Financial impact comparison
- Top priority areas and recommended actions

Note: Provider enrollment doesn't guarantee appointment availability, and financial scenarios are estimates.

Goal: Use these numbers to identify where patients may lose healthcare access and where IEHP should intervene first.

Reinventing the Care Passport — IEHP Digital Care Bridge

Instead of creating a new patient record system, IEHP should **use its existing patient data to make the transition to FFS seamless.**

How it works:

1. **Reuse existing records:** IEHP already has patient information, including medications, medical history, and care plans.
2. **Connect existing systems:** Use California's Data Exchange Framework to securely share available records with authorized FFS providers.
3. **Automate the transfer:** When patients transition, essential medical information is prepared and made accessible to their new providers, with minimal paperwork.
4. **Ensure continuity:** Help patients connect with an FFS doctor who can continue their existing treatment.

Why this is better:

- No need to build an entirely new medical records database.
- Reduces duplicate paperwork and administrative costs.
- Makes switching healthcare systems easier for patients and doctors.
- Builds on existing infrastructure instead of starting from scratch.

Our proposal: Transform the Care Passport into a **Digital Care Bridge** that connects existing healthcare systems rather than creating another one.

Implementation would require secure data-sharing agreements, privacy safeguards, and verification of what records IEHP can actually share.

Main idea: Patients should change healthcare systems, not have to restart their healthcare journey.

Reinventing the Care Passport — Universal Digital Health Profile

Main Idea: Every patient should have a digital health profile that follows them across healthcare providers and insurance systems, including IEHP and FFS.

Instead of creating new medical records every time someone changes providers, **all authorized healthcare providers should be able to securely access and update the same patient information.**

How it works:

1. **Existing Records:** Use medical information patients already have instead of starting from scratch.
2. **Connected System:** Link healthcare providers through a shared, secure health information network.
3. **Automatic Updates:** When a patient visits a doctor, their records can be updated and securely shared with other authorized providers.
4. **Seamless Transition:** When patients switch from IEHP to FFS, their medical history, medications, and treatment plans remain accessible.

Why this matters:

- Eliminates unnecessary duplicate paperwork and records.
- Prevents important medical information from getting lost during transitions.
- Improves communication between healthcare providers.
- Makes healthcare more convenient, continuous, and efficient.

Implementation: Build on California's existing health data exchange infrastructure rather than requiring every provider to adopt an entirely new system.

Our goal: Patients should have **one continuous medical history**, regardless of which doctor, hospital, or insurance system they use.

Data sources organized with overlapped rows together

ID	Source	Link
A	Medi-Cal Members by ZIP Code	Open source
B	Medi-Cal Members by County, Age, Language, and Plan	Open source
C	IEHP Fact Sheet	Open source
D / FI-1	Medi-Cal Managed Care Enrollment Report by Plan and County	Open source
E / FI-10	Medi-Cal Fee-for-Service Providers	Open source
F	Dental FFS Providers and Safety Net Clinics	Open source
G / FI-2	HRSA Health Professional Shortage Areas	HPSA tool · Download data
H	ZIP Code-to-County Crosswalk	Open source
FI-3	U.S. Census QuickFacts	Open source
FI-4	U.S. Census / data.census.gov — Full ACS Tables	Open source
FI-5	IEHP Medi-Cal Policy Change Page	Open source

FI-6	DHCS Immigration Status and Changes to Medi-Cal Eligibility	Open source
FI-7	IEHP Finance Committee Meeting Packet, October 14, 2025	Open PDF
FI-8	DHCS Fee-for-Service Transition FAQ for Providers	Open PDF
FI-9	KFF Medicaid-to-Medicare Fee Index	Open source
FI-11	City/Town Population — U.S. Census Bureau	Open source

Summary of ts:

Core analysis: A, B, D/FI-1, E/FI-10, H, FI-7
Policy/background support: FI-8, plus possibly FI-5 and FI-6
Optional context: Census, HPSA, dental, KFF, and city population sources

These are just explanation of notebook on Source A (assisted w/ H) “IEHP_Source_A_MediCal_ZIP_Analysis.ipynb”

Limitation Briefly: a lot of the information obtained from this is to be used as a baseline, knowing where ppl based on zipcode does not directly correlate to who exactly is being affected by the switch, it gives us a reference point of where the current state of things are.

A: Medi-Cal members by ZIP code “Calendar Years 2020 to the Most Recent Reportable Month”
H: ZIP code to County Crosswalk (used only Quarter 2, 2026 ; might need to expand to other years starting 2020)

Source A — Medi-Cal Membership
Tells us how many Medi-Cal members live in each ZIP code.

Source H — ZIP-to-County Crosswalk
Tells us which county each ZIP code belongs to, or overlaps with.

Combined result:
can estimate how many Medi-Cal members live in each county, identify the ZIP codes with the largest populations, and track changes over time.



San Bernardino & Riverside = the 2 primary counties IEHP serve

One technical detail:

Some ZIP codes cross county boundaries. The notebook uses the percentage of residential addresses in each county to estimate how many members belong to each county. These are estimates, not exact county membership counts.

MAIN RESULTS:

September 2026 · Estimated visible Medi-Cal eligibility in both counties
1,805,023 Members, approx

- **Riverside County**
  - 909,513
  - 50.4% of the two-county estimate

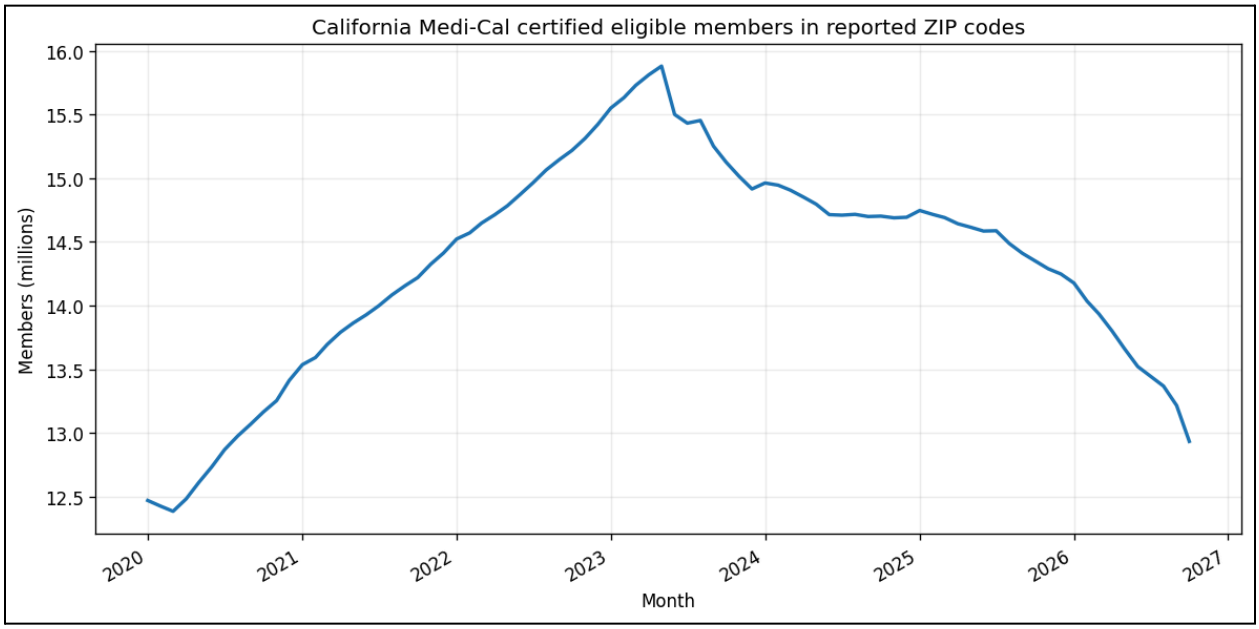
- **San Bernardino County**
  - 895,510
  - 49.6% of the two-county estimate

The estimated Medi-Cal populations of both counties are **relatively similar**. Riverside has about 14,000 more members in this dataset.

IEHP is **NOT** losing 1.8 million members. These numbers represent overall Medi-Cal eligibility, not UIS-affected members, and they include people who may belong to other health plans.

The September 2026 figures are also preliminary, and some small ZIP-level counts are hidden. Treat the figures as approximate geographic baselines.

Graph: California Medi-Cal Enrollment Trend

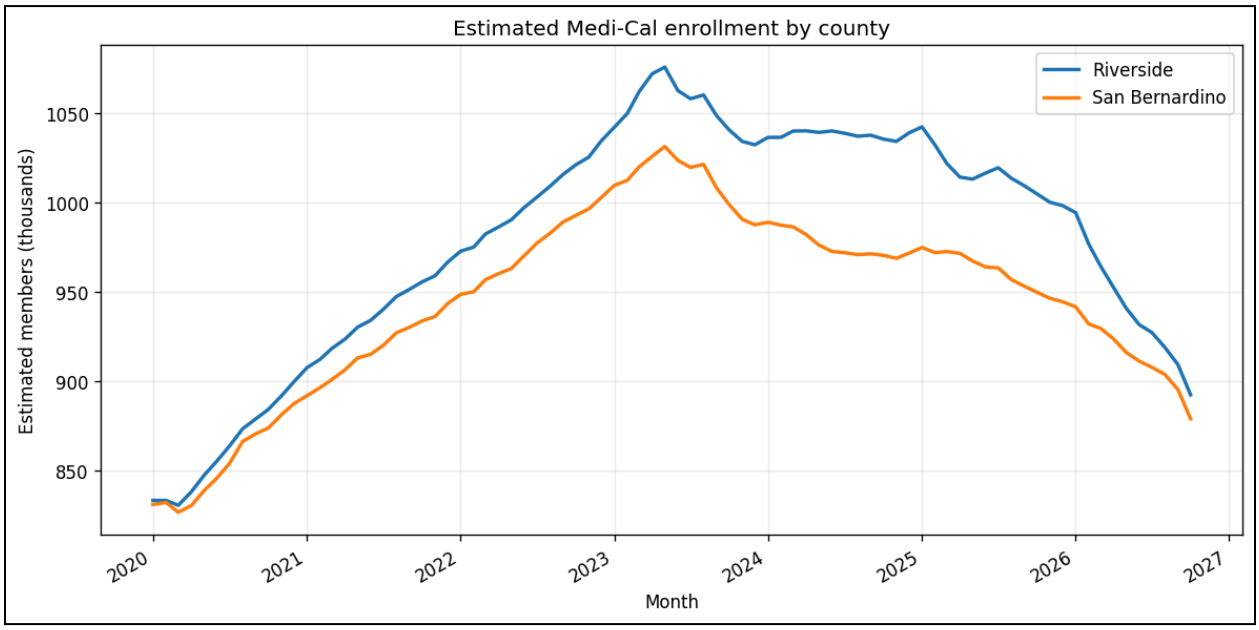


DESC: A line showing how California's total reported Medi-Cal enrollment changes from January 2020 through October 2026.

shows enrollment increasing until approximately May 2023, followed by a sustained decline.

Why this matters: It gives us background context. Medi-Cal enrollment is declining across California, so changes in Riverside and San Bernardino may reflect a broader statewide pattern. We should not assume that a decrease in Medi-Cal enrollment was caused by the upcoming FFS transition. The graph does not establish why enrollment changed.

Graph: Riverside vs. San Bernardino Enrollment Over Time

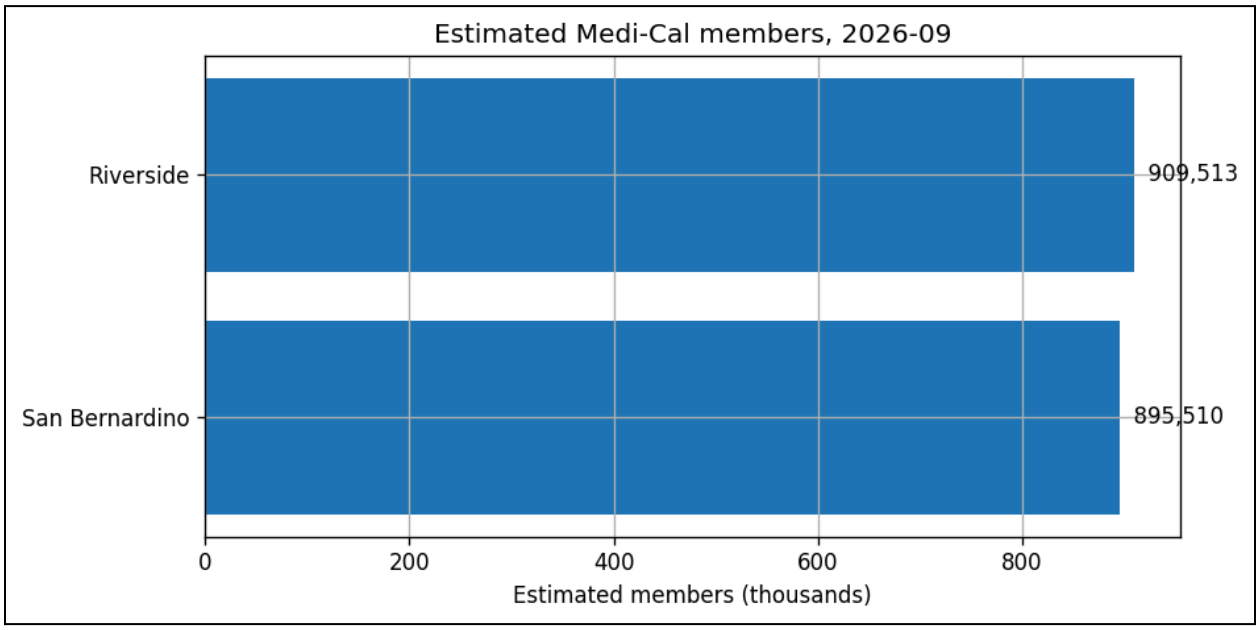


DESC: Two lines, one representing Riverside and the other San Bernardino. This is similar to Graph 1, but focuses specifically on IEHP's service area.

Both counties follow a similar pattern: enrollment rose through 2023, then declined.

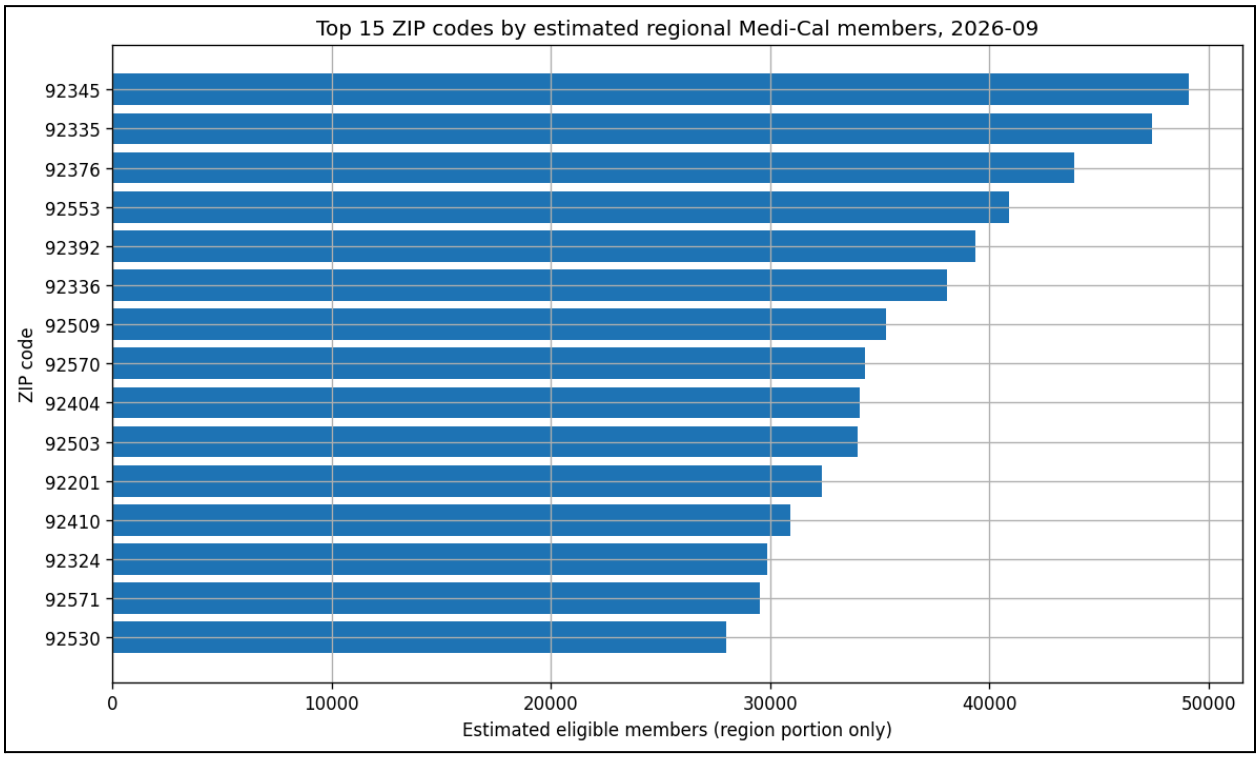
Why this matters: Riverside experienced a larger percentage decline than San Bernardino over this period. For IEHP, this suggests a question worth investigating: Are changes in the Medi-Cal population different across communities, and do those changes affect where IEHP should place its resources? This graph does not show people switching to FFS. It shows changes in overall Medi-Cal eligibility.

Graph: County Comparison



Basically just re up of prev graph and conclusion: IEHP should examine both counties rather than focus only on the county with more members.

Graph: Top 15 ZIP Codes by Medi-Cal Enrollment



Instead of asking how many Medi-Cal members live in each county, graph asks where enrollment is concentrated within those counties.

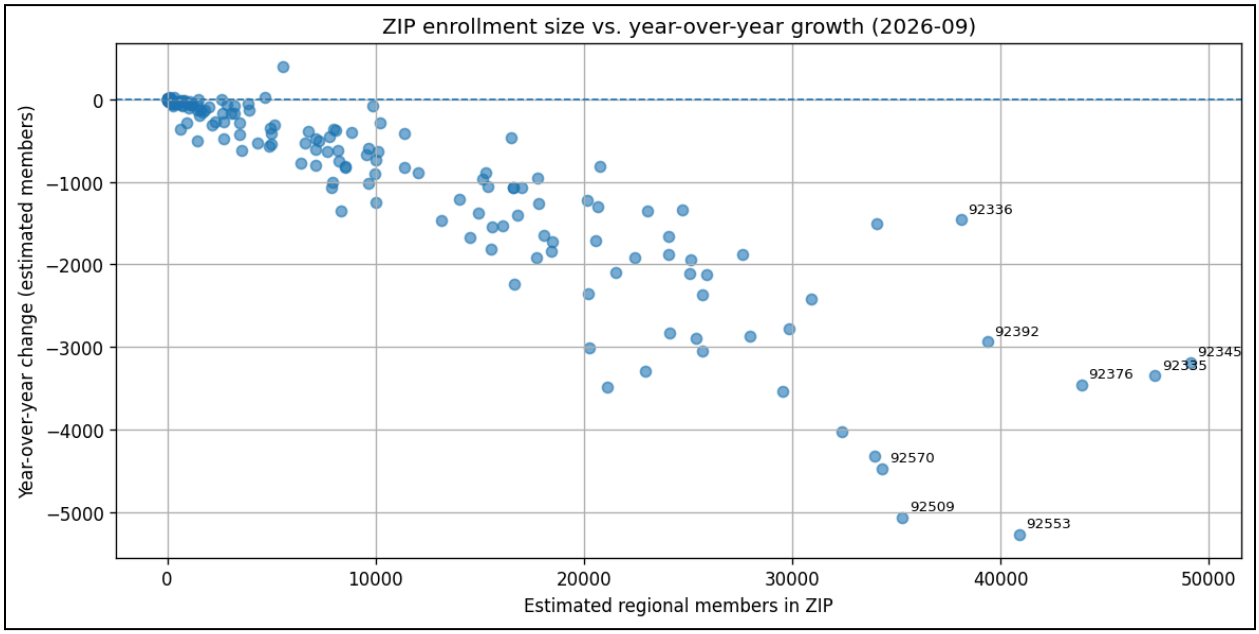
The three largest ZIP codes are San Bernardino

notebook also found that the top 10 ZIP codes contain approximately 22% of the region's estimated visible Medi-Cal membership.

Why IEHP cares: If IEHP needs to conduct outreach, investigate provider availability, or help members understand their new coverage, knowing where large populations live is a useful starting point.

For example, ZIP 92345 could be a sensible location to investigate first because it has a large overall Medi-Cal population. But we cannot call it a high-risk ZIP yet because we don't know how many of its 49,099 members are actually affected.

Graph: ZIP Enrollment Size vs. Year-over-Year Change



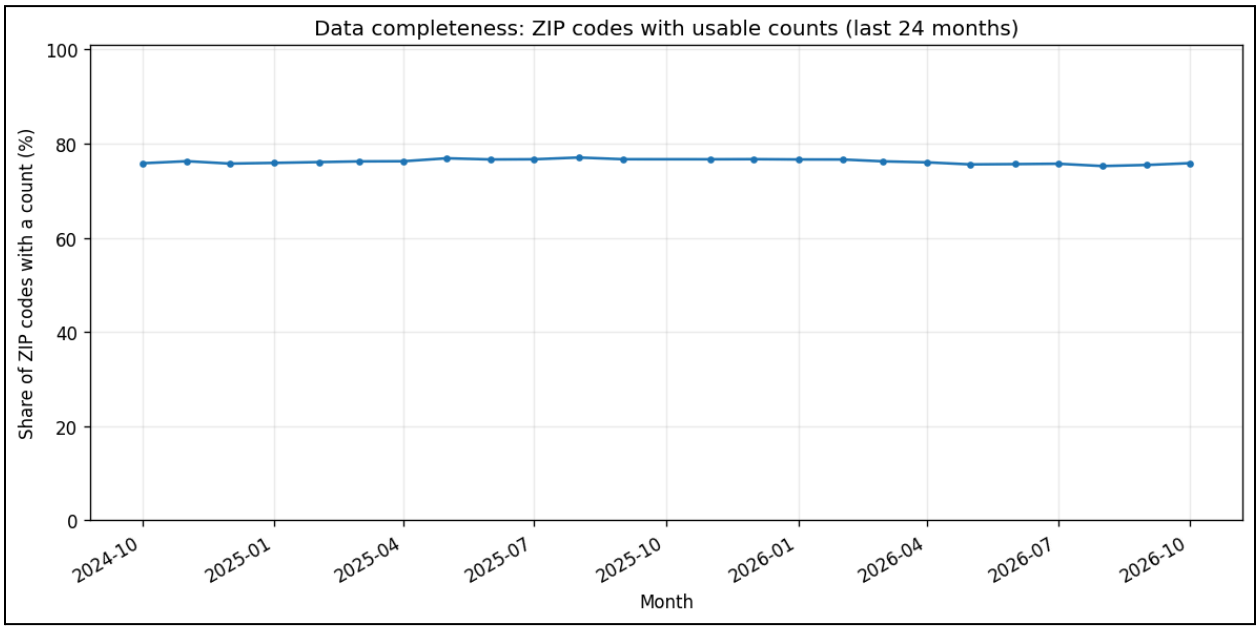
How to read it:
- Horizontal axis: How many Medi-Cal members live in the ZIP code.
- Vertical axis: How many more or fewer members that ZIP has compared with September 2025.
- Each dot: One ZIP code.

A dot farther to the right means more Medi-Cal members. A dot below zero means fewer members than the previous year. A dot above zero means enrollment increased. For example, ZIP 92553 has approximately 40,891 members in September 2026. That is 5,271 fewer members than September 2025, a decline of 11.4%.

What it reveals: Enrollment declined in the overwhelming majority of ZIP codes with usable comparisons.

Why IEHP cares: It helps identify which communities have experienced the largest changes. Those communities may deserve additional investigation, especially if affected members still need access to care. However, decreasing Medi-Cal enrollment does not necessarily mean a community has worse healthcare access.

Graph: Data Completeness



This graph measures the quality and completeness of the dataset itself. Some ZIP codes have very small member populations. Their counts are hidden for confidentiality.

	county	mapped_zip_codes	usable_zip_counts	zip_count_coverage_pct
158	Riverside	101	96	95.05
159	San Bernardino	112	98	87.50

In Riverside and San Bernardino combined, the data is more complete: 185 of 204 ZIP codes, approximately 90.7%, have usable counts.

Why IEHP cares: We need to explain how reliable our estimates are. This graph helps prevent us from presenting incomplete data as perfectly accurate.

Findings

1: Enrollment is declining.
Overall visible Medi-Cal enrollment in the two counties decreased by approximately 8% between September 2025 and September 2026.

2: Riverside and San Bernardino have similar enrollment sizes.
Neither county overwhelmingly dominates the region's Medi-Cal population.

3: Enrollment is concentrated in certain ZIP codes.
The 10 largest ZIP codes account for approximately 22% of the region's estimated visible Medi-Cal members.

Important limitation: We still don't know which members will transition.
The data includes all Medi-Cal members, not just those with UIS. Therefore, it cannot identify actual transition exposure or predict access disruptions.